

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC SitRep N°011/MVB_26/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (13)	ITURI : Aru, Bunia, Kilo, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma, Oicha, Kalunguta and Katwa SOUTH KIVU : Miti-Murhesa
Reporting date	May 25, 2026
Publication date	May 26, 2026

							
106	12*	998	238	ND	ND	0	ND
Total confirmed cases	Total deaths among confirmed cases	Cumulative number of suspected cases	Total Suspicious Deaths	Occupancy rate among suspects	Occupancy rate among experienced staff	Healed	Cumulative confirmed cases of CTE

* These deaths are underestimated because confirmation of diagnoses is delayed.

0. PROMINENT POINTS

- No samples were analyzed in Ituri.
- The unavailability of ambulances for patient transfers (only 1 ambulance available at Today) ;
- Delay in the construction of CTEs in all sites (Nyankunde, CME, HGR Rwampara);
- Resistance to the implementation of 3 EDS in the Mongbwalu health zone by some families;
- Affected provinces: **ITURI, NORTH KIVU and SOUTH KIVU**
- Cumulative confirmed cases as of May 25, 2026: 106 cases, including 94 in Ituri, 11 in North Kivu, and 1 in South Kivu. • ITURI: epicenter with 7/36 health zones affected (Aru, Bunia, Kilo, Rwampara, Mongbwalu, Nizi, and Nyankunde)
- NORTH KIVU: 5/35 ZS affected (Goma, Butembo, Oicha, Kalunguta and Katwa)
- SOUTH KIVU: 1/34 ZS affected (Miti-Murhesa)
- Of the 12 confirmed deaths, 7 were male (6 over 15 years old and 1 under 15) and 5 were female (3 over 15 years old and 1 under 15).
- Among the confirmed deaths, six were PPL

I. CONTEXT

Ituri Province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements.

has an estimated population of over 8 million inhabitants, including over one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction sites that attract people from other territories within the province and from neighboring provinces.

II. HISTORY OF THE EPIDEMIC

On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following the validation of these alerts, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy Rapid Intervention Teams (RITs) to Mongbwalu and Rwampara, which had reported suspected deaths of viral hemorrhagic fever among patients admitted to the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

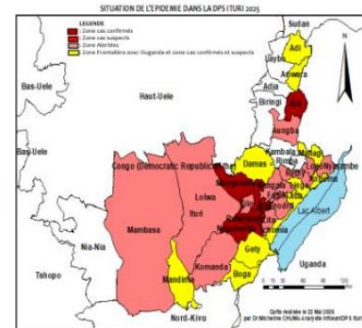
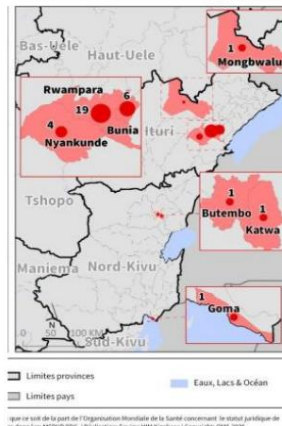
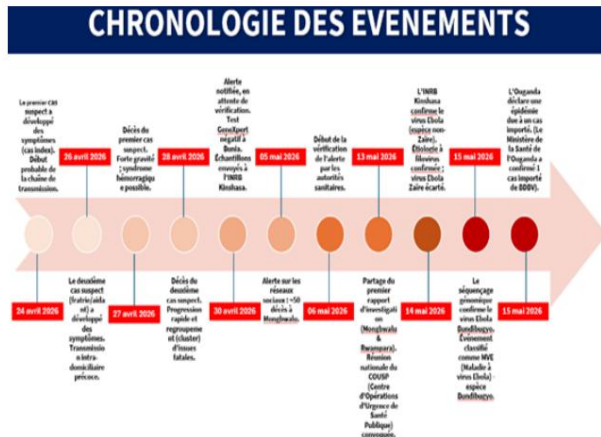
Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, *Yersinia pestis*, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, 13 of these 20 samples were tested, and 8 were found to be positive for non-Zairean Ebola virus disease at the INRB Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak in the Democratic Republic of Congo.

As of May 17, 2026, in consultation with member states, the WHO Director-General considers that the event taking place in DRC and Uganda meets the criteria of the definition set out in Article 1 of the IHR, and declared that this constitutes a Public Health Emergency of International Concern while specifying that the risk at the global level is low but high at the regional level.

On May 18, 2026, the Director General of the Africa CDC officially declared a Continental Security Public Health Emergency (CSPHE) under Article 3 of its statutes. This declaration followed an emergency meeting of the Africa CDC Advisory Group, which determined that the worsening outbreak posed an acute risk to continental health security. Consequently, the Continental Management Support Team

incident response has been activated in order to provide support to affected Member States and those exposed to risks, in terms of response and preparedness.



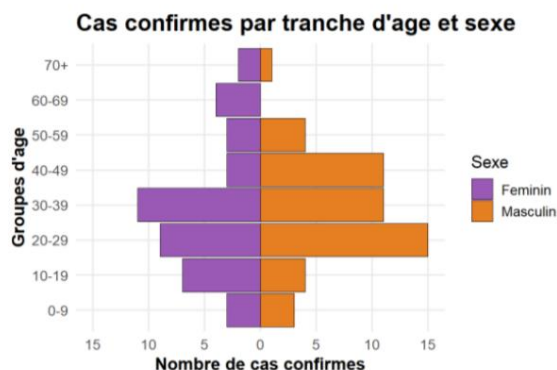
III. RESPONSE ACTIONS IMPLEMENTED SEES

III.1. COORDINATION

- Holding a strategic meeting with the Governor;
- Community dialogue around the Mayor of the rural commune of Mongbwalu.

III.2. EPIDEMIOLOGICAL SURVEILLANCE

- Carrying out field surveillance activities (*reporting alerts, investigations, active search, listing and monitoring of contacts, retrospective documentary review of cases and deaths, etc.*).
- Harmonization of monitoring data with the planning team (database)
- Finalization of the terms of reference for briefing service providers and community agents in 8 health zones and deployment multidisciplinary teams in health zones



- The 20–29 age group has the highest number of confirmed cases, especially among men.
- The 30–39 year and 40–49 year groups are also heavily affected.
- Men generally have more confirmed cases than women in **several** age groups, particularly 20–29 years and 40–49 years

Table II. Distribution of suspected and confirmed cases, deaths of Ebola virus disease in the health zones of the Ituri, North Kivu and South Kivu DPS as of May 25, 2026

Provinces	Health zones	Number of suspected cases	Number of suspicious deaths	Number of cases Confirmed	Number of contacts*
Ituri	Aru	4	1	1	0
	Bamboo	6	2	0	183
	Bunia	270	55	24	404
	Kilo	18	0	1	0
	Mongbwalu	364	88	19	222
	Nizi	9	2	2	22
	Nyankunde	66	15	9	365
	Rwampara	240	75	32	621
	Samples without a datasheet	0	0	6	0
North Kivu	Butembo	7	0	3	22
	Goma	0	0	1	28
	Katwa	13	0	4	99
	Karissibi	ND	0	0	249
	Kyondo	ND	0	1	16
	Oicha	ND	0	1	0
	Kalunguta	0	0	1	0
South Kivu	Miti Murhesa	1	0	1	ND
Total		998	238	106	2231

*This data is subject to change after harmonization of data at different levels of the health pyramid with the incident management system.

Point of Entry and Point of Control (PoE/PoC) Surveillance:

- Activations of 2 PoCs: Airport, Dele, Mudzibala, Chai and Iga-barrier and Nizi with IOM support.

Table III. Summary of PoE/PoC data as of 25/05/2026

VARIABLES	TOTAL
Travelers who have used the PoE/PoC	13,221
Screened travelers	13,075
Travelers who have washed their hands	12,005
Travelers made aware of Ebola virus disease	13,075
Listed contacts	0
Alerts raised	0



Activation of Nizi's PoC

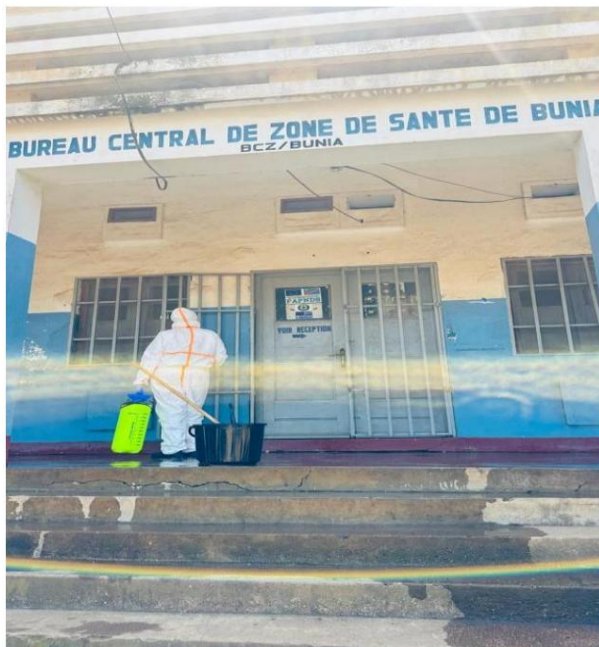
IV.3. LABORATORY

• 203 unanalyzed samples brought back to Kinshasa

Samples	Total
Samples received	431
Re-sampling (control)	3
Total samples analyzed	295
Positivity rate	35.6%
Cumulative positives	106

IV.4. INFECTION PREVENTION & CONTROL

• Decontamination of the BCZ/Bunia, the Emergency Room of the HGR-Bunia and the DPS facilities



Decontamination of the BCZ Bunia and the HGR Bunia

III.5. HOLISTIC CARE

- o Development of recovery plans for the continuity of essential health services at the Bunia General Referral Hospital, Rwampara and CME;
- o Psychological support for 7 affected households;
- o Emotional debriefing of 34 family caregivers, 23 healthcare caregivers and 15 hygiene specialists;
- o Monitoring of work at the CME (compaction completed, circuit not set up, signage not done);
- o Comparison of the lists of patients who have no results and the lab database;

IV.6. CREC

- o 45 leaders of religious denominations and churches in Ituri pledge to actively support the response against Ebola virus disease, particularly through raising awareness among the faithful and overcoming community resistance;
- o Communication on risks in schools for students and teachers in the Zones of health of Bunia at EP Kabengele (248) and Nizi at EP LUTCHAY and EP BERUNDA (276):
- o Community engagement of the 216 transporters and motorcycle taxis through awareness messages during a series of educational talks in groups at different parking lots including Kapa parking lots and Central Market roundabout and ISP (ZS Bunia).



IV.7. LOGISTICS

- Receipt of 36,000 liters of fuel with UNICEF support • WFP:
Support to the supply chain in terms of delivery, storage and transport of the various kits to the areas
- Follow-up with UNICEF for the official reception of all the donated motorcycles (the official handover was scheduled for today at the Governorate; we hope the logistics team will be in possession of these vehicles tomorrow).
;
- Monitoring of infrastructure installation work.

IV.8. SECURITY

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm

- Security briefing for newly deployed teams.
- Monitoring of the incident (isolation fire at HGR Mongbwalu).

IV. CHALLENGES

- Community resistance to the decontamination of their households and the implementation of EDS (ZS) Mongwalu and Rwampara)
- Delay in the construction of CTEs in all sites (Nyankunde, CME, HGR Rwampara);
- Need for a nursery at the Bunia General Hospital; • Insufficient supplies (PPE, medicines, etc.);
- Delay in delivering results which impacts the small isolation capacity;
- Demotivation of staff.
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V. PERSPECTIVES

- Rapidly deploy a multidisciplinary team in the Mongbwalu health zone as well as in other health zones touched
- Training of community health workers on Ebola virus disease and SBC surveillance in affected health zones and high risk in the province of ITURI.

FIELD PHOTOS



Activation of Poc Iga-Barrire



Implementation of 'EDS' on 2 alert cases



Communication about risks in schools for the benefit of students and teachers: ý s)

**FOR ALL
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